MedLingua

4-minute pitch script · runtime 4:00

> **The one rule:** the slides are for their eyes, you are for their ears. Never read a slide aloud. If a number is on screen, say what it *means* instead. Everything in brackets is a stage direction — never spoken. A line on its own is delivered alone, then you stop.

1 · Hook

0:00–0:12

> *Walk to center. Stop. Hands at your sides. Two full seconds of eye contact before you speak. The silence is the hook.*

Good morning.

Nearly thirty million people in this country can't fully understand their own doctor.

So somebody else has to speak for them. Usually somebody who should never have been in the room.

2 · The problem

0:12–0:42

> *Your emotional beat. Slow on the girl. "Un-know" is the hardest sentence in the pitch — land it, then stop. Eyes on the judges the whole time; the slide reads itself.*

A nine-year-old girl stood in an exam room and translated her mother's cancer diagnosis.

She didn't know the words. She guessed.

[beat]

And she will never un-know what she heard that day.

That is not a rare story. Twenty-nine million Americans have limited English proficiency — and when something goes wrong in their care, it goes worse. Forty-nine percent of harm events involving these patients caused physical injury. For English speakers, thirty. Half trace straight back to communication.

> *Don't say the citation. It's on the slide.*

3 · Why it isn't fixed

0:42–1:00

> *Speed up. Three quick strikes. This section should feel impatient.*

So why hasn't anyone fixed this?

A professional interpreter runs forty-five to a hundred and fifty dollars an hour, often on three-hour minimums.

So staff grab whoever's standing there — a daughter, a son. Unsafe, illegal, and now a child is carrying her mother's diagnosis.

And Google Translate? Ask it for the word "drug."

[beat]

"Droga." That's narcotics. Not medicine. And whatever you typed is sitting on somebody's server.

> *Say "droga" in Spanish. Let it hang one second. This is the line the room remembers — do not step on it.*

4 · MedLingua

1:00–1:18

> *Everything changes here. Problem voice off, builder voice on. Pick up the phone.*

This is MedLingua. Any phone becomes a medical interpreter.

Paid platforms exist. They bill by the minute, you have to schedule them, and they record the call.

We're free, we're instant, and we keep nothing. No account. No transcript. No recording. Built for the patient who reads neither language — she just taps where it hurts.

5 · Live demo

1:18–2:18

> *Start the video, then STEP TO THE SIDE. Don't stand in front of it. Say nothing. Watch them watching it. Sixty seconds of confident silence beats sixty seconds of talking over your own demo.*

6 · Under the hood

2:18–2:49

> *Highest-value thirty seconds. Turn to the screen, point at each stage as you name it. Turn back to the judges for the last line.*

Here's the pipeline, because this is where most tools stop thinking.

Speech in — and the model gets primed with clinical vocabulary in each language, reviewed by a licensed physical therapist, so it hears "lisinopril," not "listen april."

Then two gates most tools don't have. **If it isn't confident it heard you, it refuses to translate and asks you to repeat** — a wrong dose said confidently is worse than no answer. And the take-home summary gets fact-checked against the actual transcript. If the model invents a medication nobody said, the summary is thrown out and regenerated.

And underneath all of it — the audio file is deleted the second the turn ends. The transcript lives in memory and dies with the app. The summary self-destructs in two minutes.

[beat]

There is no database. There is nothing to breach.

7 · Why it works

2:49–3:07

> *Slow all the way down. This is your proof. Deliver it to one judge's face.*

And we're not guessing that this helps.

One hospital put interpreter phones at every bedside. Thirty-day readmissions for these patients dropped.

[beat]

Then they took the phones out. It went back up.

Access is the variable. So we took away its price — and its paper trail.

8 · Feasibility

3:07–3:40

> *Flat, matter-of-fact, zero salesmanship. Confidence here is quietness. Hit "they just get less" and pause.*

So how is it free? First, understand who pays today.

The clinic does — and by law they can't bill the patient for it. So when budgets get tight, they ration. They'll book an interpreter for the cancer consult, not the diabetes follow-up. The patient never gets a bill. **They just get less.**

A translated visit costs us cents. An interpreter costs a hundred dollars an hour.

And because we store nothing, there's no data to secure, no breach surface, and no medical record to maintain. **That isn't only a privacy stance — it's the reason this can be free.**

And to be straight with you: we haven't piloted yet.

Nothing is stopping us but a clinic saying yes.

> *Say the "haven't piloted" line looking right at them. Volunteering your own gap reads as confidence.*

9 · Where we are

3:40–3:49

> *Three beats, nine seconds, from memory. Don't elaborate, don't add a fourth thing.*

Seven languages on real devices, with clinician-reviewed clinical vocabulary. A summary that fact-checks itself before a patient ever sees it. And zero data stored — by design, not by policy.

10 · Close

3:49–4:00

> *Step forward. Nothing in your hands. Last two lines to their faces, not the screen. Then STOP — no "so, yeah," no "any questions."*

Healthcare has a language problem.

It's already required by law to be solved. It isn't.

[beat]

Give us one clinic and one month, and we'll show you what care sounds like when it's understood — and when nobody else is listening.

Thank you.

Eye contact map

The rubric scores this directly. These slides carry themselves — don't look back at them:

- **Slide 2** — the numbers read themselves. All thirty seconds on the judges.
- **Slide 4** — one sentence on screen. Never turn around.
- **Slide 5** — the video is the visual. Stand beside it and watch *them*.
- **Slide 7** — your proof line goes to a person, not a wall.
- **Slide 8** — especially "we haven't piloted." Looking away turns honesty into apology.
- **Slide 10** — eyes up the entire close.

Slide 6 is the only one worth turning to. Point at each stage as you name it, then come back for "nothing to breach."

If a judge pushes back

"How is this different from Google Translate?"
"Google Translate gives you 'droga' for 'drug,' and it keeps what you typed. We prime the model with clinical vocabulary, and when it isn't confident it heard you, it refuses rather than guesses. And we store nothing at all."

"What if the AI gets it wrong?"
"Two things catch it. Low confidence means it won't translate — it asks you to repeat. And the take-home summary is fact-checked against the transcript, so an invented medication never reaches the patient. When it still can't get through, the app shows them their legal right to a free human interpreter, on screen, in their language."

"What happens to patient data?"
"The audio file is deleted when the turn ends. The transcript never touches disk. The summary self-destructs in two minutes. There's no account and no database — the only thing we send is a random install ID for rate limiting."

"Isn't sending audio to OpenAI a privacy problem?"
"It's processed and discarded, and the app tells the patient exactly that in their own language on first launch, before anything is sent. That disclosure is on screen — I can show you."

"Who validated the medical terms?"
"A licensed physical therapist reviewed the clinical vocabulary — anatomy, pain, and symptom language especially."

"How do you stay free?"
"A visit costs cents in model usage, and because we store nothing there's no compliance infrastructure to fund. A club budget or one small grant covers a clinic for a year."

"Why hasn't a clinic piloted it?"
"We built it first. We wanted something real to put in their hands instead of a pitch. That's the ask today."

Before you walk on

- Drop the demo recording into slide 5. An empty placeholder costs you the whole prototype category.
- Your name and email on slide 10.
- **Pick one number** — your slide says 29.6M, your website says 27.6M.
- **Be ready to name the hospital in slide 7.** "One hospital" invites "which one?"
- Have the app open on the phone before you start, in case a judge asks to hold it.
- Memorize slides 1, 9, and 10 word for word. Opening and closing are what they remember.
- Run it twice against a timer. If you run long, cut slide 3 to two sentences — never trim 6 or 7.
